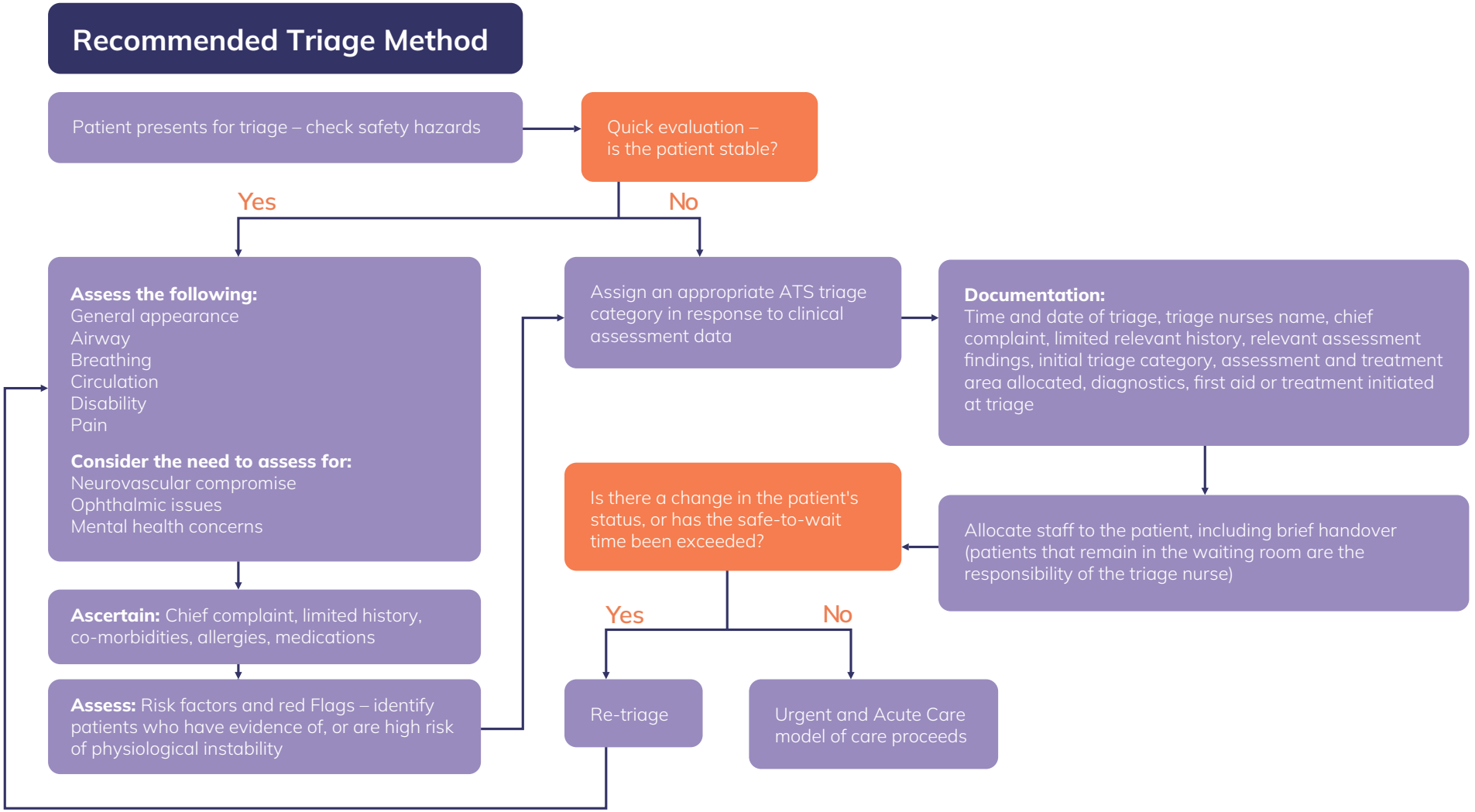
Triage Tool

Urgent Care Clinic
Australasian Triage Scale ATS
Quick Reference Guide

ace hub



Triage Process Flowchart



ATS categories for treatment acuity and performance thresholds

ATS Category	Treatment Acuity (maximum waiting time for assessment and treatment by RN or/and NP/DR)	Performance Indicator
1	Immediate	100% of the time
2	10 minutes	80% of the time
3	30 minutes	75% of the time
4	60 minutes	70% of the time
5	120 minutes	70% of the time

DNW Policy Triage

Background and statement

Person-centered care promotes recognition, understanding, and consideration of each person's unique needs and circumstances, including culture, beliefs, values, traditions, family situations, social circumstances, lifestyle, and preferences. This approach supports and encourages each person to fully participate in decision-making as much as possible or desirable in their own care which may mean they choose to leave without care.

However, Doctors, Registered Nurses and Paramedics retain a duty of care for all patients and the follow-up of patients that DNW should be adhered to in accordance with this policy. To reduce the risk of adverse outcomes. Especially in a patient with high risks and /or poor health literacy or with a question around competence.

Documentation

If a patient is not present in the waiting room when called three times, this is to be documented on the DNW form below at the time of calling the patient.

DNW	Responsible clinician	Date/ Time	Signature	Action
Patient not found/ Present after three calls	Caller			
Charge Nurse or associate charge nurse notified	CNM or ACNM signs			
Doctor or Medical director notified if applicable	Doctor or Medical Director signs			

DNW Policy Triage cont.

Process

The patient is called three times at least 10 minutes apart. If not found, all three DNW episodes are to be documented in the patient management system (PMS). Following the third patient call, enter the DNW details. The staff member who called the patient notifies the triage nurse/charge nurse manager or associate charge nurse manager, who then ensures the DNW is entered into the appropriate patient database and is to request the Patient's Medical Record for review (if available).

The most senior RN (as discussed above) is to then review the information recorded at triage and subsequently the nature of the patient's documented presenting complaint and is to check the Patient Management System (PMS) for any history of DNW

The most senior RN is to consider the risk to the patient of DNW with particular consideration of:

1. the ATS triage score and assessment including HR, Sp02, RR and pulse characteristics
2. whether the patient is paediatric (in general under 18 years)
3. children identified as having or requiring a report of concern
4. Whether the patient has presented previously but DNW or did not complete their assessment on one or more prior presentations
5. whether the patient is in a high-risk demographic / vulnerable population
6. Any patient presenting with mental health concerns or has a known mental health history (confirmed via the medical record) is to be considered, at a minimum, a moderate risk under the definition of this policy
7. Any patient presenting with deliberate self-harm, suicidal ideation or thoughts to harm others are to be considered high risk under the definitions of this policy.

Once the senior RN has assessed the patients risk as a result of their DNW status, they are to discuss the risk assessment with the relevant staff for escalation purposes for medium and high risk patients

DNW Policy Triage cont.

Risk assessment:

When it comes to DNW Patient Risk Assessment It is acknowledged that there is no perfect predictor for knowing what low, moderate and high risk is. Therefore, risk assessment will rely on your best clinical assessment each time. Low risk has been defined as requiring no action, moderate risk requires deferred action and high risk requires immediate action.

Low Risk – No Action where the senior RN determines that the risk to the patient is low, no further action needs to be taken and this decision and supporting evidence is to be entered into the form above and signed.

Moderate Risk – Deferred action where the senior RN or doctor determine a moderate risk to the patient who did not wait, the patient is to be referred for active follow up by way of phone call during the next available business hours. This follow up may be by the senior RN or through handover to another senior RN during a normal business hours shift. Where the patient is engaged with services, social work, or other appropriate persons, contact can be attempted through these means. In these cases, active efforts are to be undertaken to make phone contact with the patient and encourage them to return for assessment and if not to contact their GP and the conversation is to be documented in the medical record. Inability to contact the patient is to be documented and further risk assessment made by the senior RN in collaboration with a doctor.

High Risk – Immediate action where the senior RN and the doctor determine that there is a high immediate risk to the patient who did not wait to be seen. The senior RN is to immediately attempt contact with the patient in an effort to encourage them to return. The mental health services or other appropriate support can be enlisted if appropriate and available, especially for vulnerable patients such as elderly patients and/or paediatric patients. Where these attempts at contact are unsuccessful but the patient is still considered to be at significant risk, request for assistance to locate the patient is to be escalated appropriately to police. All actions and conversations are documented in the patient's medical record.

Special considerations:

Special consideration for elderly, paediatrics, mental health patients and vulnerable patients, person-centred care should focus on the social, emotional and cultural needs of the person and ensure patients and families/carers are involved in patient's care. Person centred care should recognise the diversity of people and communities including across gender, age, language, geographic location, sexual orientation, religious beliefs, family responsibilities, life experiences and educational levels.

DNW Policy Triage cont.

Definitions:

Did not wait (DNW) Patients who present to Clinics / Services and did not wait to be treated after triage are classified as 'did not wait' for reporting purposes.

Senior (RN) Most senior nurse on shift.

Records Management:

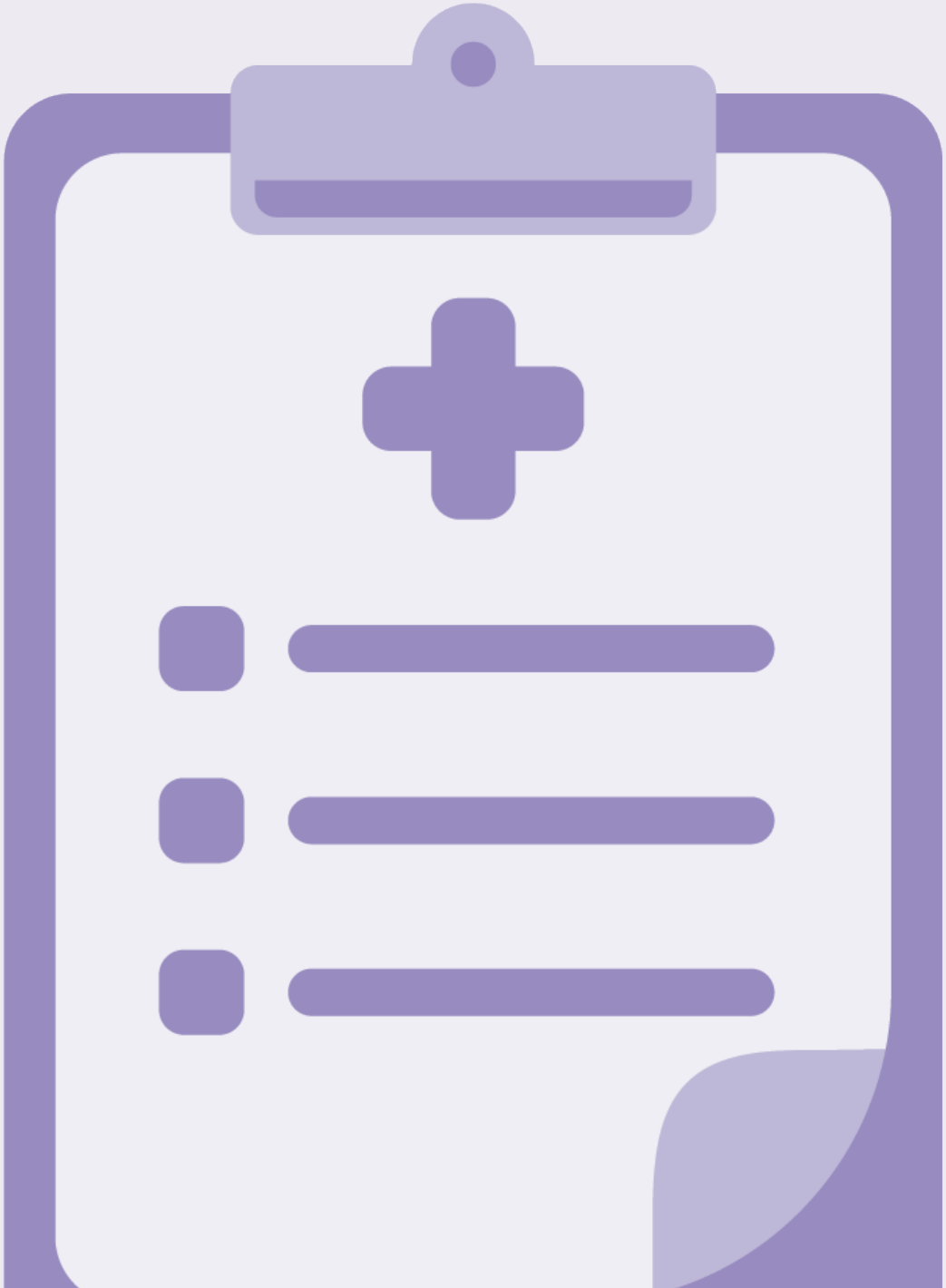
All medical records/ notes must be stored in the approved PMS and Records Management System in accordance with PMS Policy. All clinical records must be managed in accordance with patient privacy and confidentiality.

The Senior Doctor or senior RN are to review DNW statistics and follow up outcomes weekly. To monitor for DNW trends, raising any unwanted clinical variation with those with operational accountability and supporting formulation and documentation of any clinical risks or improvement plans. Review of this procedure is to be carried out every three to five years from implementation of the procedure.

References:

1. .Australian Government Department of Health and Ageing. (n.d.). Triage Education Kit. Retrieved September 2022, from Australian College for Emergency Medicine: https://acem.org.au/getmedia/c9ba86b7-c2ba-4701-9b4f-86a12ab91152/Triage-Education-Kit.aspx
2. Committee, N. E. (Amended 2015, Amended March). Mental Health Ed Guide. Retrieved from NSW Health: https://www.health.nsw.gov.au/mentalhealth/resources/Publications/mental-health-ed-guide.pdf

This course is based on the Australasian Triage Scale and The Standard 2015. This content has been adapted to meet the industry needs of Urgent Care and PHC in New Zealand



Adult Triage

Adult Triage Tool

Criterion	Category 1 Immediate	Category 2 10 minutes	Category 3 30 minutes	Category 4 60 minutes	Category 5 120 minutes
Airway Risk ATS 1 Potential Risk ATS 2 No Risk ATS 3-5	– Obstructed – Immediate airway risk	– Partially obstructed – Anaphylaxis – Drooling w distress – Severe stridor – Potential airway risk	– Patent – No airway risk	– Patent – No airway risk	– Patent – No airway risk
Breathing Severe Resp Distress ATS 1 Moderate Resp Distress ATS 2 Mild Resp Distress ATS 3 No Resp Distress ATS 4-5	– Severe respiratory distress/absent respiration/ hypoventilation – RR <10 – Central cyanosis – Either severe or no use of accessory muscles	– Moderate respiratory distress – Peripheral cyanosis – Speaking in words – Moderate use of accessory muscles	– Mild respiratory distress – Speaking in short sentences 3-4 words per breath – Minimal use of accessory muscles	– No respiratory distress – No SOB – No WOB – Speaking full sentences	– No respiratory distress – No SOB – No WOB – Speaking full sentences
Circulation Severe Shock ATS 1 Moderate Shock ATS 2 Mild Shock ATS 3 No Shock ATS 4-5	– Severe haemodynamic compromise/absent circulation – Uncontrolled haemorrhage – Cardiac arrest – Symptomatic – BP < 80mmhg (no radial pulse)	– Moderate haemodynamic compromise – Non symptomatic – BP <80mmgh (no radial pulse)	– Mild haemodynamic compromise	– No haemodynamic compromise	– No haemodynamic compromise
Circulation GCS or AVPU	– GCS <9 – Unresponsive or responds to pain	– GCS 9–12 – Responds to pain	– GCS >12 – Responds to pain or voice	– Normal GCS 15 – Alert	– Normal GCS 15 – Alert
Pain Objective and Subjective Assessment		– Severe Pain – 7/10 - 10/10	– Moderate Pain – 4/10 – 6/10	– Mild Pain – 1/10 – 3/10	– No Pain
Neurovascular Colour, Warmth, Movement and Sensation		– Severe neurovascular compromise – Pulseless, cold, altered sensation, delayed peripheral cap refill – Dislocations	– Moderate neurovascular compromise – Weak pulse present, cool, altered sensation, delayed peripheral cap refill	– Mild neurovascular compromise – Normal pulse, warm, mild altered sensation, normal peripheral cap refill	– No neurovascular compromise

Adult Vital Signs

	Heart Rate	Resp Rate	SBP	Cap Refill	Temp	Sp02
Normal Adult	60 - 100 bpm	12 - 20 bpm	90 - 120	< 2 sec	36.5 - 37.5c	> 94% ra
COPD Adult	60 - 100 bpm	Increased RR	90 - 120	< 2 sec	36.5 - 37.5c	88 - 92% ra

Reference : National EWS Chart

Levels of Compromise: Circulation

Severe Haemodynamic Compromise **Severe Shock ATS 1**	Moderate Haemodynamic Compromise **Moderate Shock ATS 2**	Mild Haemodynamic Compromise **Mild Shock ATS 3**	No Haemodynamic Compromise **No Shock ATS 4-5**
The patient is severely compromised if their HR, RR, Cap Refill, GCS and Pulse are markedly abnormal such that there is a high risk of cardiac arrest, unless immediate interventions are carried out. Severe hypotension, or absent radial pulses or an inability to obey commands, or severe shortness of breath. The assessment of the level of cardiovascular compromise can be further validated by a visual assessment of the patient. 'looks ATS 1' the patient is usually severely compromised. Pale, diaphoretic and cold with grossly abnormal to RR and levels of GCS	The patient is moderately compromised if their RR, GCS/ AVPU, HR, Pulse and Cap Refill are abnormal or significant symptoms of MI are present. For example, when the patient has: hypotension or prolonged capillary refill time, or an altered level of consciousness with an ability to obey commands, or moderate shortness of breath. The assessment of the level of cardiovascular compromise can be further validated by a visual assessment of the patient. 'looks ATS 2' the patient is usually moderately compromised. Pale, diaphoretic, cold.	Mildly compromised if their RR, Pulse, Peripheral Cap Refill, normal GCS / AVPU, are near to normal and / or mild symptoms of MI are present. The assessment of the level of cardiovascular compromise can be further validated by a visual assessment of patient. 'looks ATS 3' the patient is usually mildly compromised. May be pale, dry and cool.	Not compromised if their HR, RR, Pulse and Peripheral Cap Refill are normal and there are no symptoms of MI present. The assessment of the level of cardiovascular compromise can be further validated by a visual assessment of the patient. 'looks ATS 4 - 5' the patient is usually not compromised. Warm, dry and well perfused.

Levels of Compromise: Breathing

Severe Respiratory Distress / Absent Respiration ATS 1	Moderate Respiratory Distress ATS 2	Mild Respiratory Distress ATS 3	No Respiratory Distress ATS 4 - 5
– Extremely short of breath, or hypoventilation < 10bpm – Unable to speak – May or may not have adventitious lung sounds – Severe or absent use of accessory muscles – Severe agitation or decreased LOC – Rapidly falling Sp02 – (Hypoventilation with absent adventitious lung sounds and no use of accessory muscles is a very late sign – imminent respiratory arrest) – Central and peripheral cyanosis	– Very short of breath – Peripheral cyanosis – Speaking in only words per breath – May only have quiet adventitious lung sounds – Significant chest / neck indrawing – Tripod position – May be agitated – Sp02 < 90% ra – Moderate use of accessory muscles	– Mild respiratory distress – Speaking in short sentences 3-4 words per breath – Minimal use of accessory muscles – May have loud adventitious lung sounds – Normal LOC – Normal Sp02 for patient	– No SOB, No WOB

The ABCs of Obstetrics

Urgency	Urgency Indicator	Reason for Caution
Airway	- Any potential compromise - Increased nasal congestion	Often difficult intubations due to: • Increased patient size • Difficulty with patient positioning • Different induction agents required - Increased vascularity of nose and airways causes difficulty in breathing
Breathing	- Asthma	- Progesterone may be responsible for increased drive to breathe - One third of pregnant asthmatic women experience a deterioration in their condition
Circulation	- Palpations - Headaches - Sudden drop in BP - Symptoms of pulmonary embolus	- Progesterone causes widespread vasodilatation - Oestrogen may contribute to increases in blood volume - Diastolic BP - 16-17mmHg decrease - BP lowest during the second trimester - Cardiac output (CO) – by 30-50% increase - Hyperdynamic flow - High volume and dynamic flow may cause cerebral hemorrhage, especially in subarachnoid hemorrhage (SAH) during pregnancy - Sudden and serious deterioration of their condition - Changes in coagulation system associated with pregnancy

Points to Remember

- Hyperdynamic and physiological changes occur as early as 6-8 weeks gestation.

- An assessment of urgency must be made on the basis of both the woman and the foetus.

- An elevated BP is an ominous sign: the higher the BP the more urgent the review.

- At 20 weeks the weight of the uterus compresses the inferior vena cava if the woman is lying on her back - a compromise to foetal wellbeing.

- The risk of many conditions is higher in pregnant women than non-pregnant women of childbearing age.

- These conditions include:
  - ceberal hemorrhage or cerebral thrombosis
  - severe pneumonia
  - atrial arrythmias
  - venous thrombosis
  - chlolelithiasis
  - pyelonephritis
  - spontaneous arterial dissections
    e.g. splenic and subclavian dissections, with no previous medical history.

- Domestic violence is more prevalent during pregnancy. This can mean increased complications for mother and adverse neonatal outcomes.

- In the setting of trauma, maternal signs may remain stable even when loss of one-third of blood volume may have occurred.

- The best initial treatment for the foetus is the optimum resuscitation of the mother.

Normal Vital Signs in Pregnancy

Vital Sign	First Trimester	Second Trimester	Third Trimester
Systolic Blood Pressure mmHg	94.8 to 137.6	95.6 to 136.4	101.6 to 143.5
Diastolic Blood Pressure mmHg	55.5 to 86.9	56.8 to 87.1	62.4 to 94.7
Heart Rate Beats Per Minute	63.1 to 105.2	67.4 to 112.5	64.5 to 113.8
Respiratory Rate	8 to 24	8 to 24	8 to 24
Oxygen Saturation SpO2 (%)	94.3 to 99.4	92.9 to 99.3	93.4 to 98.5
Temperature	35.55 to 37.51	35.35 to 37.37	35.37 to 37.35

Reference : Green LJ, et. al,Gestation-Specific Vital Sign Reference Ranges in Pregnancy. Obstet Gynecol. 2020 Mar;135(3):653-664. PMID: 32028507

Pain Assessment: Numerical Scale 0-10

Validated Methods for Quantitative Assessment of Pain

Visual analogue scale: 100mm line

Nelson, Cohen, Lander, et al 2004

Use 100mm line as shown



Ask the patient to mark their level of pain on the line.

Abbey Pain Scale: For measurement of pain in people who cannot verbalise

How to use scale: While observing the patients, score questions 1 to 6

Q1. Vocalisation
eg: whimpering, groaning, crying

Absent 0 Mild 1 Moderate 2 Severe 3

Q2. Facial Expression
eg: looking tense, frowning, grimacing, looking frightened

Absent 0 Mild 1 Moderate 2 Severe 3

Q3. Change in Body Languange
eg: fidgeting, rocking, guarding part of body, withdrawn

Absent 0 Mild 1 Moderate 2 Severe 3

Q4. Behavioural Change
eg: increased confusion, refusing to eat, alteration in usual patterns

Absent 0 Mild 1 Moderate 2 Severe 3

Q5. Physiological Change
eg: temperature, pulse or blood pressure outside normal limits, perspiring, flushing or pallor

Absent 0 Mild 1 Moderate 2 Severe 3

Q6. Physical Changes
eg: skin tears, pressure areas, arthritis, contractures, previous injuries

Absent 0 Mild 1 Moderate 2 Severe 3

Add scores 1-6 and record here. Total Pain Score =

Now tick the box that matches the Total Pain Score:

0-2 No Pain	3-7 Mild	8-13 Moderate	14+ Severe

Finally, tick the box that matches the type of pain:

Chronic	Acute	Acute on chronic



Paediatric Triage

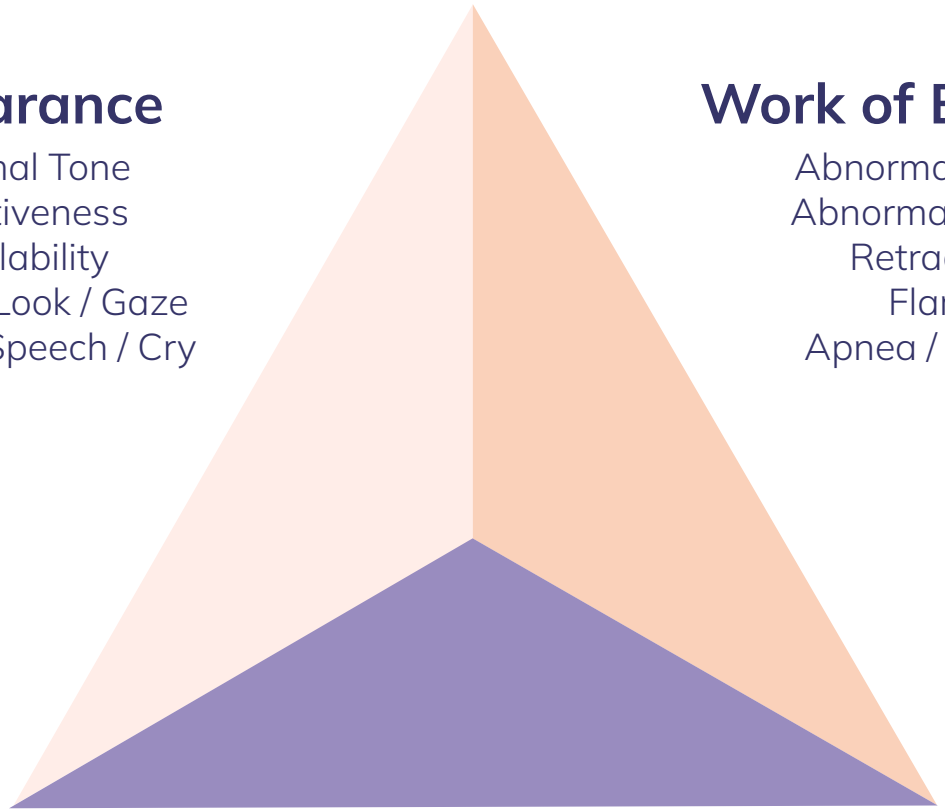
PAEDIATRIC ASSESSMENT TRIANGLE

Appearance

Abnormal Tone
↓ Interactiveness
↓ Consolability
Abnormal Look / Gaze
Abnormal Speech / Cry

Work of Breathing

Abnormal Sounds
Abnormal Position
Retractions
Flaring
Apnea / Gasping



Circulation to Skin

Pallor
Mottling
Cyanosis

Normal Paediatric Vital Sign Ranges: By Age Group

Age	Heart Rate	Respiratory Rate	SBP	Cap Refill	Temperature	Sp02
< 3 months	110 - 150 bpm	30 - 60 bpm	60 - 80 mmHg	< 2 sec	36.5 - 37.5c	> 94% ra
3 - 12 months	100 - 150 bpm	25 - 50 bpm	80 - 100 mmHg	< 2 sec	36.5 - 37.5c	> 94% ra
1 - 5 years	90 - 120 bpm	20 - 40 bpm	90 - 110 mmHg	< 2 sec	36.5 - 37.5c	> 94% ra
5 -12 years	70 - 110 bpm	20 - 30 bpm	90 - 120 mmHg	< 2 sec	36.5 - 37.5c	> 94% ra
> 12 years	60 - 100 bpm	12 - 17 bpm	100 - 130 mmHg	< 2 sec	36.5 - 37.5c	> 94% ra

Reference : CMDHB, Kids First, Bedside PEWS Score

Paediatric Triage Tool

	ATS 1	ATS 2	ATS 3	ATS 4	ATS 5
Airway **Red Flags = unable to verbalise, no cry, drooling, difficulty swallowing, stridor!**	– Obstructed – Partially obstructed with severe respiratory distress – Severe stridor – Drooling	– Patent – Partially obstructed with moderate respiratory distress – Biphasic Stridor Minor drooling Severe Croup	– Patent – Partially obstructed with mild respiratory distress – Croup	– Patent	– Patent
Breathing WOB = nasal flaring, grunting, subclavian retractions, tracheal tug, intercostal indrawing, abdo breathing, head bobbing **Red Flags = grunting, head bobbing, pallor/ cyanosis, agitated / inconsolable, floppy / lethargic! pre-terminal = floppy, cyanosed, bradycardic!**	– Absent respiration or hypoventilation – Severe respiratory distress, e.g. • Severe use accessory muscles • Severe retraction • Acute cyanosis – Not speaking **Extreme / Absent WOB**	– Respiration present – Moderate respiratory distress, eg. • Moderate use accessory muscles • Moderate retraction • Skin pale – Speaking single words per breath **Severe WOB**	– Respiration present – Mild respiratory distress, e.g. • Mild use accessory muscles • Mild retraction • Skin pink. – Speaking short sentences **Moderate WOB**	– Respiration present – No respiratory distress • Mild use accessory muscles • No retraction, no abdo breathing, no nasal flaring. – Speaking full sentences **Mild / No WOB**	– Respiration present – No respiratory distress • No use accessory muscles • No retraction. – Speaking full sentences **No SOB / No WOB**
Circulation Assess peads for - tachy or brady, skin colour, cap refill, bleeding, dehydration (wet diapers, feeding, dry mucosa)	– Absent circulation or significant bradycardia e.g. HR <60 in an infant (that looks unwell) – Severe haemodynamic compromise, e.g. absent peripheral pulses skin pale, cold, moist, mottled significant tachycardia capillary refill >4 secs. **Uncontrolled hemorrhage**	– Circulation present – Moderate hemodynamic compromise, e.g. weak/threadybrachial pulseskin pale, cool, moderate tachycardia capillary refill 3 – 4 secs. – Severe signs of dehydration **> 6 s/s dehydration**	– Circulation present – Mild haemodynamic compromise, e.g. palpable peripheral pulse, skin pale, warm mild tachycardia. cap refill < 2 secs – Moderate signs of dehydration – Less wet diapers, Hx of gastro illness, dry mucosa, sunken fontanelle **3–6 s/s dehydration**	– Circulation present – No haemodynamic compromise, e.g. palpable peripheral pulses skin pink, warm, dry. – Mild signs of dehydration normal cap refill **< 3 s/s dehydration**	– Circulation present – No haemodynamic compromise, e.g. palpable peripheral pulses – Skin pink, warm, dry. – Normal cap refill **No s/s dehydration**

Paediatric Triage Tool continued...

	ATS 1	ATS 2	ATS 3	ATS 4	ATS 5
Disability AVPU	**GCS < 8** – Ongoing seizure • Flacid • Tonic – clonic • Tonic only seizure. – Responds to pain or unconscious	**GCS 9–12** – Severe decrease in activity, e.g. • no eye contact, • decreased muscle tone • responds to voice or pain	**GCS > 13** – Moderate decrease in activity, e.g. • lethargic • eye contact when disturbed. • responds to voice	**Normal GCS (15) or no acute change to usual GCS.** – Mild decrease in activity, e.g. • quiet but eye contact • interacts with parents. • alert	**Normal GCS (15) or no acute change to usual GCS.** – No alteration to activity, e.g. • playing • smiling. • alert
Pain Assessment Wong Baker Faces FLACC		**Severe Pain 8-10/10** e.g. – patient/parents report severe pain – skin, pale, cool – alteration in vital signs – requests analgesia.	**Moderate Pain** e.g. – patient/parents report moderate pain – skin, pale, warm – alteration in vital signs – requests analgesia.	**Mild Pain** e.g. – patient/parents report mild pain – skin, pink, warm – no alteration in vital signs – requests analgesia.	**No or Mild Pain** e.g. – patient/parents report mild pain – skin, pink, warm – no alteration in vital signs – declines analgesia.
Neurovascular Assessment Use play to assess status		**Severe Neurovascular Compromise** e.g. – pulseless – cold – nil sensation – nil movement – capillary refill.	**Moderate Neurovascular Compromise** e.g. – pulse present – cool – sensation – movement – capillary refill.	**Mild Neurovascular Compromise** e.g. – pulse present – normal / sensation – normal / movement – normal capillary refill.	**No Neurovascular Compromise**

Assessment of Dehydration Levels in Infants

SIGNS	SEVERITY		
	Mild	Moderate	Severe
General Condition	Thirsty, restless, irritable	Thirsty, restless, agitated	Withdrawn, somnolent or comatose; rapid deep breathing
Pulse	Normal	Rapid, weak	Rapid, weak
Anterior Fontanelle	Normal	Sunken	Very sunken
Eyes	Normal	Sunken	Very sunken
Tears	Present	Absent	Absent
Mucous Membranes	Slightly dry	Dry	Dry
Skin Turgor	Normal	Decreased	Decreased with tenting
Urine	Normal	Reduced, concentrated	None for several hours
Weight Loss	4-5%	6-9%	>10%

FLACC Behavioural Pain Scale

	0	1	2
Face	No particular expression or smile	Occasional grimace or frown, with-drawn, disinterested	Frequent to constant quivering chin, clenched jaw
Legs	Normal position or relaxed	Uneasy, restless, tense	Kicking or legs drawn up
Activity	Lying quietly, normal position, moves easily	Squirming, shifting back and forth, tense	Arched, rigid or jerking
Cry	No cry (awake or sleep)	Moans or whimpers; occasional complaint	Crying steadily, screams, sobs, frequent complaints
Consolability	Content, relaxed	Reassured by touching, hugging or being talked to, distractible	Difficult to console or comfort

Instructions:

Patients who are awake:
- Observe for at least 2-5 minutes
- Observe legs and body uncovered
- Reposition patient or observe activity, assess body for tenseness and tone
- Initiate consoling interventions if needed

Patients who are asleep:
- Observe for at least minutes or longer
- Observe body and legs uncovered.
- If possible reposition the patient.
- Touch the body and assess for tenseness and tone.

Each category is scored on a 0-2 scale which results in a total score of 0-10

Assessment of Behavioural Score:

0 = Relaxed and comfortable
1 - 3 = Mild discomfort
4 - 6 = Moderate pain
7 - 10 = severe discomfort/pain

Reference: Merkel S, Voepel-Lewis T, Shayevitz JR, et al: *The FLACC: A behavioural scale for scoring postoperative pain in young children.* Pediatric Nursing 1997;23:293-797
Printed with permission © 2002. The Regents of the University of Michigan

Wong-Baker FACES Pain Rating Scale

Developed for young patients to communicate how much pain they are feeling.



0	2	4	6	8	10
No Hurt	Hurts Little Bit	Hurts Little More	Hurts Even More	Hurts Whole Lot	Hurts Worst

INSTRUCTIONS

Explain to the child that each face is for a person who feels happy because he has no pain (hurt) or sad because he has some or a lot of pain.

Face 0 is very happy because he doesn't hurt at all.
Face 2 hurts just a little bit.
Face 4 hurts a little more.
Face 6 hurts even more.
Face 8 hurts a whole lot more.
Face 10 hurts as much as you can imagine, although you do not have to be crying to feel this bad.

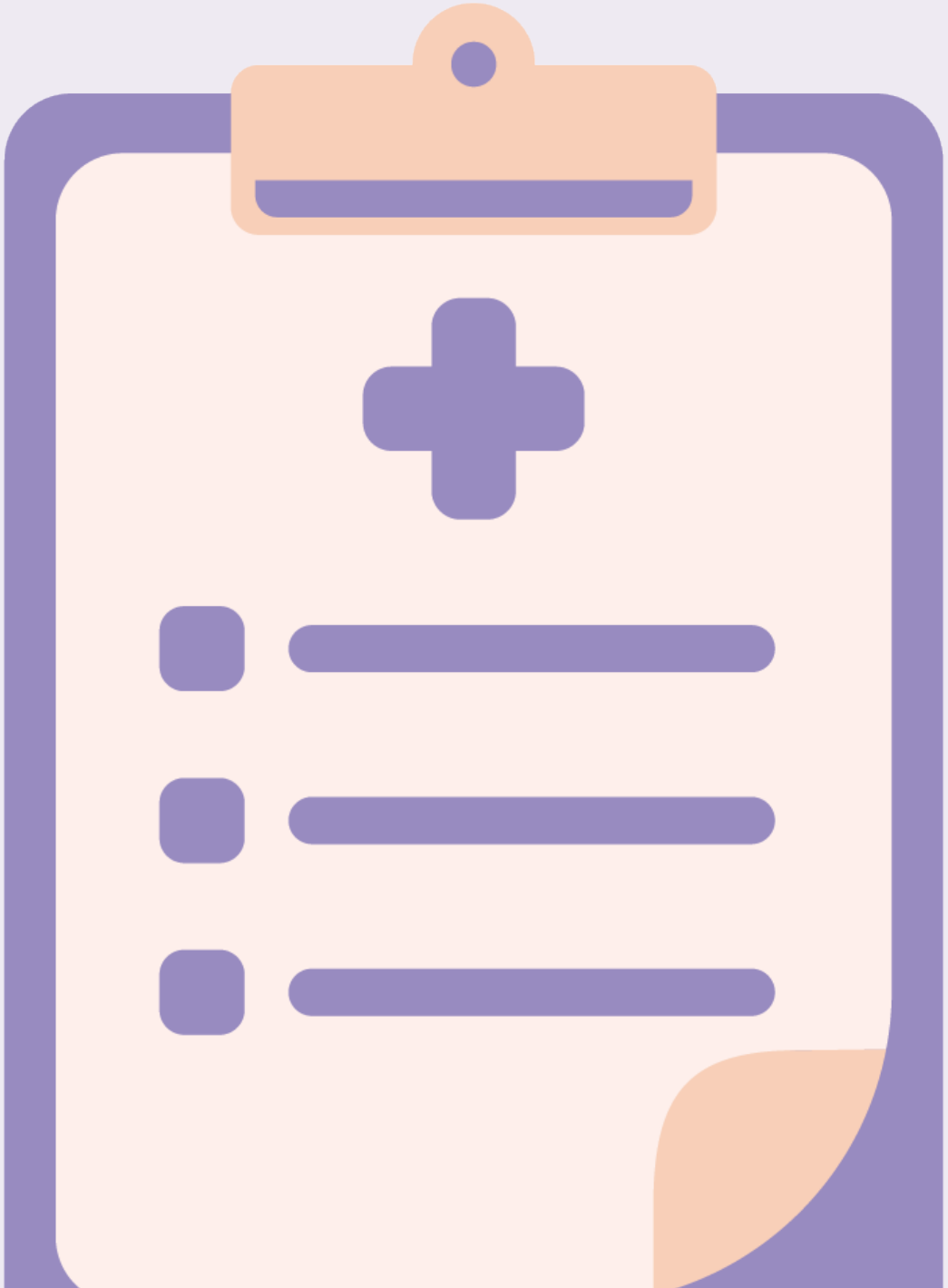
Ask the child to choose the face that best describes how he/she is feeling.

Reference: Hockenberry MJ, Wilson D, Winkestein ML: *Wong's Essentials of Pediatric Nursing.* ed 7, St. Louis, 2005, p. 1259. Used with permission. Copyright, Mosby.
©1983 Wong-Baker FACES Foundation. www.WongBakerFACES.org

Risk Factors consideration for triage Adults and Children

Paedriatric specific risks in RED

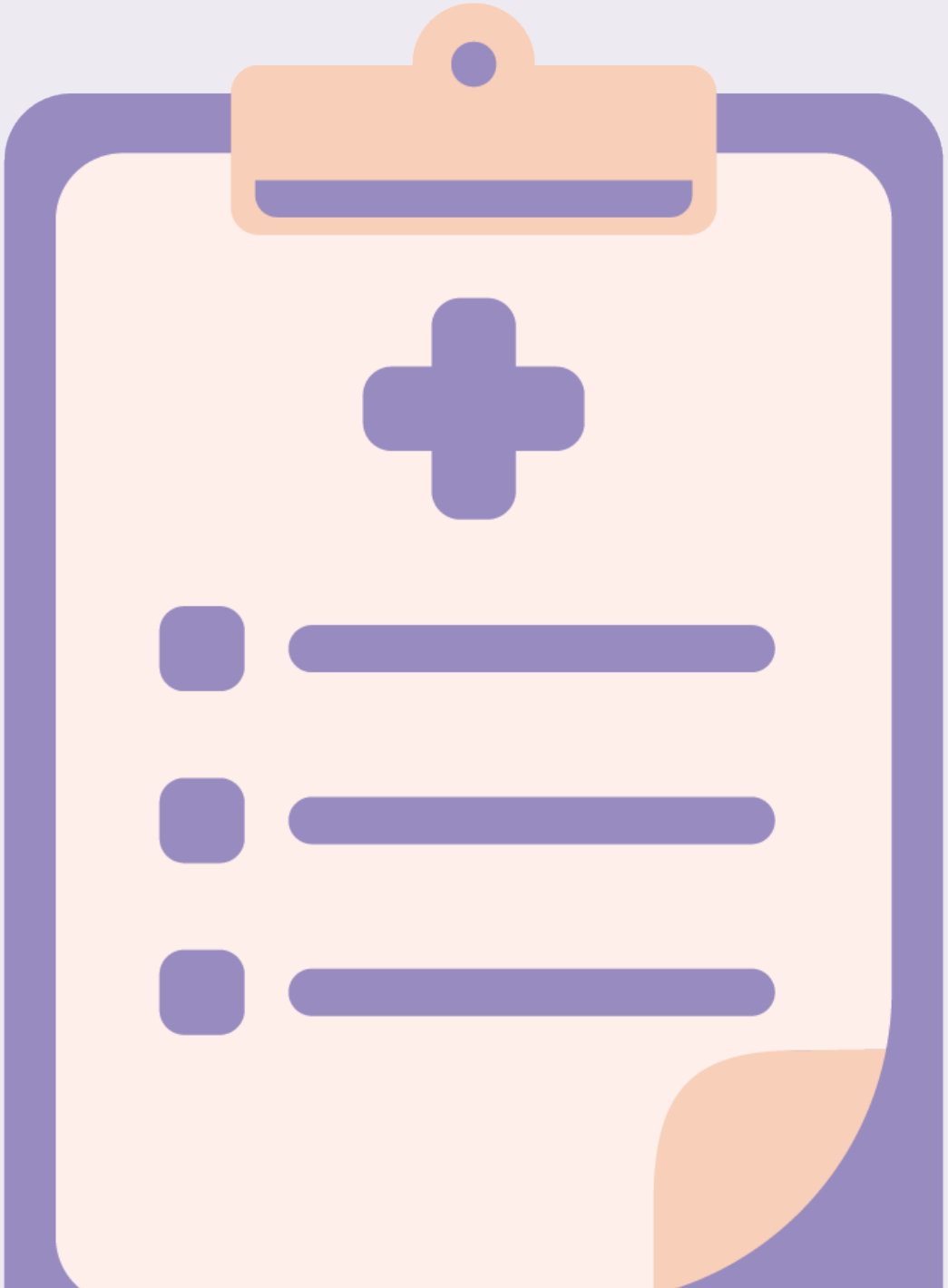
Age	Co-morbidities	Events Preceding Presentation	Mechanism of Injury (MOI)	Red Flags / Cautions
- ≤ to 65 - Poly pharmacy - Comorbidities - ≥ 28 days - Stable Neonate ATS 3 - Unstable Neonate ATS 2	- Hx prematurity graduate of Neonates - Respiratory disease - Cardiovascular disease - Renal disease - Cancer - Diabetes - Substance abuse - Immuno-compromised - Congenital disease - Complex medical Hx - Intellectual disability - Cognitive impairment - English second language - Hearing aids/sight impaired - Transplant patients - Polypharmacy - Chicken pox – Reyes Syndrome (Aspirin and Ibuprofen)	- Apnoeic/cyanotic episode - Seizure activity - Decreased intake decreased output - Red current jelly stool - Bile stained vomiting - Parental concern - Paediatric overdose - Fever in a child less then one year - Collapse vs fall - Recent illness - Fever in Adults - Hx of infectious contact	- Penetrating injury - Fall >2 x height - RTC/MVA >60 kph - MBA/cyclist > 30 kph - Pedestrian - Ejection/rollover - Prolonged extrication (>30 minutes) - Death in same car - Staring on windscreen - Compound or major # - Dislocation of large joint - Explosion - Prolonged crush - Severe localised trauma, #, dislocation, amputation, lac	- Thunder clap headache – subarachnoid bleed - Elderly patients on Anticoagulants or Betablockers - Situational Urgency (Abuse) - AAA – ABDO/BACK Pain - Torsion testicle/ ovary - Meningitis - Atypical MI - Significant sedative or intoxication, drugs/ ETOH - Acute stoke - Sepsis / Neuropenic sepsis - NAI children or adults - Paradoxical behaviours - Drooling - Toxic looking child - Significant dehydration - **Pregnancy** 3rd trimester pregnant women with headache or dizziness. Pre-Eclampsia. Ectopic. Shock may be masked. Gestation > 24 wks is considered 2 Pt's



Opthalmology Triage

Opthalmology Triage Tool

Category 1 Immediate	Category 2 10 minutes	Category 3 30 minutes	Category 4 60 minutes	Category 5 120 minutes
	- Penetrating eye injury - Chemical injury - Sudden loss of vision with or without injury - Sudden onset severe eye pain, vision change and redness - Orbital cellulitis extending to forehead - Globe rupture – tear drop pupil	- Sudden abnormal vision with or without injury - Moderate eye pain, e.g. • blunt eye injury • flash burns • foreign body - Hyphemia – bleeding into cornea & iris space - Detached retina – floaters, loss of vision in peripheral fields. - Globe rupture – tear drop pupil. - Orbital cellulitis	- Normal vision - Mild eye pain, e.g. • blunt eye injury • flash burns • foreign body	- Normal vision - No eye pain - Conjunctivitis - Bleeding in eye - Sticky or red itchy eye



Mental Health Triage

Mental Health Triage Tool

Triage Code – Treatment Acuity	Description	Typical Presentation	General Management Principles *
1 – Immediate Right Now (You would **definitely** cross the street if you saw this person)	**Definite danger to life (self or others)** **Australasian Triage Scale[1] states:** - Severe behavioural disorder with immediate threat of dangerous violence (breaking laws)	**Observed** - as seen at triage - Violent behaviour - Possession of weapon - Self-destruction in clinic - Extreme agitation or restlessness - Bizarre/disoriented behaviour **Reported** - Verbal commands to do harm to self or others, that the person is unable to resist (command hallucinations) - Recent violent behaviour	**Supervision** Continuous visual surveillance 1:1 ratio (see definition below) **Action** - Alert clinical medical staff immediately - Alert mental health triage or equivalent - Provide safe environment for patient and others - Ensure adequate personnel to provide restraint/detention based on industry standards **You will** - Call security +/- police if staff or patient safety compromised. May require several staff to contain patient - 1:1 observation - Intoxication by drugs and alcohol may cause an escalation in behaviour that requires management.

Mental illness is not often acute! Consider medical causes!

UTI/infections, head injury / TBI / bleed / stroke, hypoxia, hypoglycemia, hypothermia, ETOH / withdrawal, CNS infection, metabolic, renal failure, space occupying lesions

Mental Health Triage Tool continued...

Triage Code – Treatment Acuity	Description	Typical Presentation	General Management Principles*
2 – Emergency Within 10 minutes (You would **probably** cross the street if you saw this person)	**Probable risk of danger to self or others** *AND/OR* **Severe behavioural disturbance** **Australasian Triage Scale[1] states:** Violent or aggressive (if): - Immediate threat to self or others - Requires or has required restraint - Severe agitation or aggression	**Observed** - Extreme agitation/restlessness - Physically/verbally aggressive - Confused/unable to cooperate - Hallucinations/delusions/ paranoia - Requires restraint/containment - High risk of absconding and not waiting for treatment – Risk to self and others! **Reported** - Attempt at selfharm/threat of self-harm - Threat of harm to others - Unable to wait safely - Self harm with plan to continue self harm	**Supervision** Continuous visual supervision (see definition below) **Action** - Alert clinical medical staff immediately - Alert mental health triage - Provide safe environment for patient and others - Use defusing techniques (oral medication, time in quieter area) - Ensure adequate personnel to provide restraint/detention - Prompt assessment for patient recommended under Section 9 or apprehended under Section 10 of Mental Health Act. **You will** - If defusing techniques ineffective, re-triage to category 1 (see above) - Security in attendance until patient sedated if necessary - Intoxication by drugs and alcohol may cause an escalation in behaviour that requires management

Mental Health Triage Tool continued...

Triage Code – Treatment Acuity	Description	Typical Presentation	General Management Principles*
3 – Urgent Within 30 minutes (You would maybe cross the street if you saw this person)	**Possible danger to self or others** - Moderate behaviour disturbance - Severe distress **Australasian Triage Scale[1] states:** - Very distressed, risk of self-harm - Acutely psychotic or thought -disordered - Situational crisis, deliberate self-harm - Agitated/withdrawn	**Observed** - Agitation/restlessness - Intrusive behaviour - Confused - Ambivalence about treatment - Not likely to wait for treatment **Reported** - Suicidal ideation – with or without plan - Situational crisis - Self harm, no plan to continue **Presence of psychotic symptoms** - Hallucinations - Delusions - Paranoid ideas - Thought disordered - Bizarre/agitated behaviour **Presence of mood disturbance** - Severe symptoms of depression - Withdrawn/uncommunicative and/or anxiety - Elevated or irritable mood(mania)	**Supervision** Close observation (see definition below) - Do not leave patient in waiting room without support person **Action** - Alert mental health triage - Ensure safe environment for patient and others **Consider** - Re-triage / assessment if evidence of increasing behavioural disturbance i.e. - Restlessness - Intrusiveness - Agitation - Aggressiveness - Increasing distress - Inform security that patient is in department - Intoxication by drugs and alcohol may cause an escalation in behaviour that requires management

Mental Health Triage Tool continued...

Triage Code – Treatment Acuity	Description	Typical Presentation	General Management Principles*
4 – Semi-urgent Within 60 minutes (You would not feel the need to cross the street if you saw this person)	**Moderate distress** **Australasian Triage Scale[1] states:** - Semi-urgent mental health problem - Under observation and/or no immediate risk to self or others - Medication compliance - Feeling low/ not suicidal	**Observed** - No agitation/restlessness - Irritable without aggression - Cooperative - Gives coherent history **Reported** - Pre-existing mental health disorder - Symptoms of anxiety or deperession without suicidal ideation - Willing to wait	**Supervision** Intermittent observation (see definition below) **Action** Discuss with mental health triage nurse **Consider** - Re-triage if evidence of increasing behavioural disturbance i.e. - Restlessness - Intrusiveness - Agitation - Aggressiveness - Increasing distress - Intoxication by drugs and alcohol may cause an escalation in behaviour that requires management

Mental Health Triage Tool continued...

Triage Code – Treatment Acuity	Description	Typical Presentation	General Management Principles*
5 – Non-urgent Within 120 minutes	**No danger to self or others** - No acute distress - No behavioural disturbance **Australasian Triage Scale[1] states:** - Known patient with chronic symptoms - Social crisis, clinically well patient	**Observed** - Cooperative - Communicative and able to engage in developing management plan - Able to discuss concerns - Compliant with instructions **Reported** - Known patient with chronic psychotic symptoms - Pre-existing non-acute mental health disorder - Known patient with chronic unexplained somatic symptoms - Request for medication - Minor adverse effect of medication - Financial, social, accommodation, or relationship problems	**Supervision** General observation (see definition below) **Action** - Discuss with mental health triage - Refer to treating team if case-managed

Management Definitions
Continuous visual surveillance = person is under direct visual observation at all times Close observation = regular observation at a maximum of 10 minute intervals Intermittent observation = regular observation at a maximum of 30 minute intervals General observation = routine waiting room check at a maximum of 1 hour intervals

* Management principles may differ according to individual health service protocols and facilities.

Australasian College of Emergency Medicine (2000). Guidelines for the implementation of the Australasian Triage Scale (ATS) in Emergency Departments.

The Mental State Examination

A clinician assesses aspects of a person's mental state across the assessment process, from the initial observation of the person and while taking their history. A detailed Mental State Examination encompasses the full range of a person's expression and not just what they say. The clinician will observe a person's apparent mood, how they behave in the session, note details of their speech and how the person is responding during the interview, and in turn how the clinician is responding to the person. A thorough assessment of mental state will cover the following areas (NB Many of these overlap but have been described separately for clarity).

Appearance and general behaviour: This includes the person's facial expressions, body language, posture, gestures, manner, attire and grooming. These may give clues to the person's personality, mood, awareness of social conventions and ability to take care of themselves. A psychotic person may behave very bizarrely, or a depressed person may appear unkempt or disheveled.

Attitude towards the interviewer: This includes whether the person was suspicious or hostile, evasive, tentative, or co-operative and whether the interviewer felt some rapport had been established. It is also important for the interviewer to consider their reaction to the person. The degree of rapport or therapeutic engagement established is directly related to how willing a person will be to disclose painful or difficult material. This is crucial in assessing for suicide risk as a disengaged or obviously reluctant person may not give a full account of their risk or intent if they do not trust the interviewer or want to be taking part in the assessment process. In such circumstances, it is particularly important to give weight to information from others and to balance this against any denials of suicidal intent during the interview that appear contradictory to these reports.

Affect/mood: Affect refers to the prevailing emotional tone of the interview, and is assessed through the person's facial expression, body posture, and tone of voice. Mood is more of a prolonged feature that is reported by the person (and others if available). A clinician should look for how emotionally responsive a person is during an interview, whether they appear fl at and apathetic or at the other extreme, labile or irritable. It is also important to consider whether their reported mood matches their displayed mood (eg, someone who is speaking slowly, wringing their hands and is intermittently tearful but says they are 'fine' needs to be assessed further). Important dimensions of mood to assess for include depressed mood (eg, sad, tired etc), elated or euphoric mood, irritability and agitation. A clinician should also be aware of obvious signs of tension (eg, sweating, stammering, trembling) and reported anxiety.

Psychomotor activity: This refers to internally driven behaviours, for example, drumming of fingers that may represent feelings of anxiety. Typical behaviours that the clinician should be aware of include restlessness/agitation; slowing of movement, speech and thoughts; questions answered after prolonged delays.

Speech: A great deal of additional information can be gained from listening to how a person speaks and not just the content of their speech. The clinician should listen for the loudness and speed of speech and how effective the person is in getting their point across. For example, a person who has pressured speech may feel compelled to talk and the listener may gain the impression that their ideas flow faster than they can express them. A depressed person may demonstrate 'poverty of speech', such as responding after delays with 'I don't know', or very short or monosyllabic replies.

The Mental State Examination

Thought content: Inferences about a person's thinking are made on the basis of their report of symptoms. A careful exploration of a person's presenting difficulties and history is crucial if the clinician is to gain an accurate picture of their thinking. In assessing for suicidality a clinician will pay particular attention to suicidal ideation and depressive thoughts (eg, 'I'm no good, I'd be better off dead'). The clinician should ask the person about their beliefs about themselves, others and their place in the world, and screen for the presence of delusional beliefs, anxious thinking, obsessions etc.

Thought process: Again, the clinician makes inferences about a person's thinking based upon their speech. When assessing thought processes the clinician is looking for evidence of the way the person constructs their thoughts, including sequence and speed. Marked difficulties in thought processes may include perseveration (ie, when a person seems to be stuck on one theme), tangentiality (ie, easily side-tracked without being able to return to the initial topic), or loose associations (ie, apparent absence of logical thought processes). A clinician will also be looking at the person's style of thinking (eg, do they worry or ruminate over their misfortunes, are they preoccupied with certain topics/events/ideas? For example, a person may report negative selftalk as above, but then also report that one thought triggers another related one, and another, until they feel like they can't 'shut them off').

Perceptual disturbance: In this instance the clinician is specifically screening for the presence of hallucinations. These may occur across any of the five senses. If the person is suicidal and hearing voices, care must be taken to assess whether these voices are command hallucinations (ie, telling the person to act) and how compelling these commands are. It is also important to assess for the degree of fear associated with the experience of hallucinations as this may also increase a person's suicide risk (ie, ending one's life to escape the feared hallucination).

Orientation: Briefly, this is a screen to check that the person knows where they are, who they are and why they are being interviewed. This series of questions is especially important if the person has come into the emergency department (eg, following an overdose). If the person is disoriented to place, person or time, careful examination is needed to rule out delirium, intoxication or organic complications. Disorientation in time, especially the passage of time (such as the length of the interview), is the most sensitive of these tests of orientation.

Insight and judgment: A clinician needs to judge how much the person is aware of their present circumstances, their health and the reasons for their presentation to mental health services or an emergency department. If a person is assessed as being significantly impaired in their judgment, in a way that places them at risk of harm to themselves or others, they can be compelled to have treatment under the Mental Health (Compulsory Assessment and Treatment) Act 1992.

Intelligence/cognitive function: For the purpose of a suicide assessment a broad estimation of a person's intellectual functioning is sufficient. This can be inferred from the content of their speech, educational history and vocational background. Other aspects of cognitive functioning to screen for are attention and concentration and any memory problems. The clinician can be aided in this by using the Mini Mental State Examination.

APPENDIX 1

RAPID ASSESSMENT OF PATIENTS IN DISTRESS

These questions are to assist in the assessment of people with potential mental health problems in the emergency department, and to consider urgency of need for mental health referral and security measures.

Name:	Address:	DOB:
Next of Kin:	Telephone:	NHI:

Commentary

Use Mental State Examination if disoriented.

Consider safety issues for person and staff.

Use prompts over page to assess for suicide.

*** May need to institute measures to prevent inappropriate self discharge prior to full assessment NOW.**

Is the person too sedated, sleepy or disoriented to give an adequate history?

Does the person need an interpreter?

Enquire about old notes/any existing mental health files.

Start a problem list.

Seek consent to contact whānau/ family/friends.

Check if they are from this area.

Do you need to contact anyone immediately or can it wait until the following morning?

Fax contact information.

WHAT IS THE PROBLEM?

Disorientation in:

- ☐ Time?
- ☐ Place?
- ☐ Person?

- ☐ Suicidal ideation*
- ☐ Anxiety/panic attacks
- ☐ Thoughts disordered
- ☐ Bizarre behaviour*
- ☐ Depressed
- ☐ Intoxication*
- ☐ Hallucinating
- ☐ Aggression*
- ☐ 'Stress'
- ☐ Drug and alcohol withdrawal*
- ☐ Delusional
- ☐ Agitation*

WHY NOW?

- ☐ Self-referral
- ☐ Concern from others
- ☐ Personal crisis
- ☐ GP referral
- ☐ Person won't talk
- ☐ Mental Health Act 1992
- ☐ Police presentation
- ☐ Deliberate self-harm

CAN I GET AN ADEQUATE HISTORY NOW? ☐ Yes ☐ No

IS THERE A PSYCHIATRIC HISTORY? ☐ Yes ☐ No

If yes, working diagnosis? _____

Current medications? _____

Legal status? ☐ Mental Health Act 1992 ☐ Informal

Is there an existing management plan? ☐ Yes ☐ No

WHAT DOES THE PERSON WANT?

- ☐ Someone to talk to
- ☐ Accommodation
- ☐ They don't know
- ☐ Psychiatric input
- ☐ Medication
- ☐ Inpatient treatment
- ☐ Detox
- ☐ Outpatient treatment

WHERE CAN I GET INFORMATION FROM? (note contact details)

- ☐ Whānau/family members or friends?
- ☐ GP?
- ☐ Community health centre?
- ☐ Voluntary agencies?
- ☐ Case management?
- ☐ Therapist/counsellor?
- ☐ NGOs?
- ☐ Psychiatrist?
- ☐ Psychologist?

IS ANYONE ELSE WITH THEM OR IN THE WAITING ROOM?

Does the person have children? ☐ Yes ☐ No

Where are they now? _____

CAN YOU TELL ME HOW YOU ARE FEELING AT THE MOMENT?

Adapted from Wilhelm, Kotze, Ballard and Hudson, Consultation Liaison Psychiatry and Emergency Departments, St Vincent's Hospital, Sydney. 2001.

These questions will help determine the level of observation needed in the emergency department.

Commentary

Consider whether the person is safe to be alone.

Risk factors include:
- definite plan
- hopelessness
- severe depression
- psychotic symptoms
- recent discharge from a psychiatric unit
- use of alcohol, street drugs, particularly recent escalation
- recent suicide attempt
- single men: young, elderly
- homelessness
- medical illness
- history of childhood abuse
- recent suicide attempt by a whānau/family member or a friend.

IF YOU SUSPECT A PERSON IS CARRYING A WEAPON NOTIFY SECURITY NOW

Predictors of violence include history of impulsivity and previous violence, alcohol and substance use, antisocial traits/behaviours.

Complete a physical examination.

Make triage decisions and decide whether to refer for a psychiatric assessment.

Does the Mental Health (Compulsory Assessment and Treatment) Act 1992 need to be applied? Is a Duly Authorised Officer required?

Does Child, Youth and Family Services need to be called (to offer 'care and protection' to under 17 year old)? (0508 FAMILY)

Involve social supports in discharge planning.

SUICIDE ASSESSMENT

Have you had thoughts that life isn't worth living? ☐ Yes ☐ No

Have you thought of harming yourself? ☐ Yes ☐ No

Are you thinking of suicide? ☐ Yes ☐ No

Have you tried to harm yourself in the past? ☐ Yes ☐ No

If yes, how many times? _____

When was the most recent time? _____

How often are you having these thoughts? _____

Have you thought about how you would act on these (is there a plan)? *(Does this plan seem feasible? Are the methods available? Is it likely to be lethal?)*

_____

Have you thought about when you might act on this plan?

Are there any things/reasons that stop you from acting on these thoughts? _____

Do you know anyone who has recently tried to harm themselves?

IF A SUICIDE ATTEMPT HAS BEEN MADE

What did you hope would happen as a result of your attempt? *(Did they want to die, or end their pain?)* _____

Do you still have access to the method used? ☐ Yes ☐ No

Did you use alcohol or drugs before the attempt? What did you use?

Do you have easy access to a weapon? ☐ Yes ☐ No

HAVE YOU THOUGHT OF HURTING ANYONE ELSE? ☐ Yes ☐ No

If yes, have you acted on these thoughts? ☐ Yes ☐ No

Have you been involved in any fights recently? ☐ Yes ☐ No

If yes, were you using drugs or alcohol at the time? ☐ Yes ☐ No

DO YOU FEEL SAFE AT THE MOMENT? ☐ Yes ☐ No

WHO DO YOU LIVE WITH?

Is there anyone at home right now? ☐ Yes ☐ No

Is there anyone we need to call? _____

HOW IS YOUR GENERAL HEALTH?

WHAT TRIGGERED THESE THOUGHTS/ACTIONS?

(What are key stressors, for the person, eg, financial, relationships, employment, bereavement?)

What lead you to try and take your life? _____

What lead you to having these thoughts? _____

What has been going on in your life lately? Are there any things that have been worrying or upsetting you? _____

WHAT WOULD MAKE A DIFFERENCE AT THE MOMENT?

(Can the person see anything changing in their situation?)

Can we make a list of the main problems? _____

What can we do now? _____

What needs to be done later? _____